CODE 12 — COMMUNITY CARE, FUNCTIONAL FRAILTIES AND COMMUNITY HEALTH

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Art. 1 — Subject matter, definitions and scope of competence

1. This Code governs the organisation of community care, the taking into care of functional frailties, the management of care spaces and care instruments and the planning of the time necessary to guarantee essential services. These resources constitute infrastructural commons that serve the implementation of the Sufficiency Basket, the protection of health and access to housing.

2. For the purposes of this Code:

   a) **community care** means the set of activities of support for daily, relational and domestic life, of accompaniment, of educational support and of practical assistance that do not constitute clinical services;

   b) **functional frailty** means a temporary or permanent condition that limits the person's autonomy in performing one or more essential functions included in levels 1 and 2 of the Basket, irrespective of the presence, the absence or the classification of a clinical diagnosis;

   c) **Care Basket** means the component of the Sufficiency Basket governed by Code 1 that comprises the minimum care time, access to care spaces and, where necessary, support with meals, with hygiene, with accompaniment and with protected rest;

   d) **care spaces** means the spaces and facilities intended, according to need and territorial planning, for the kitchen supporting frail persons, the common laundry, children from zero to three years of age, protected rest, non-clinical proximity assistance and temporary community reception;

   e) **individual care plan** means the confidential document that, on the basis of the functional need ascertained and with respect for the will of the person, identifies the services necessary, the parties responsible, the expected duration and the dates or conditions of review;

   f) **Care Safety Sheet** means the operational document that identifies the risks connected with a specific care activity and establishes the preventive measures, the competences required, the interruption procedures and the arrangements for substitution in the event of unavailability of an essential resource;

   g) **Care STV** means the binding technical specification, adopted in accordance with Code 2, that defines the standards, minimum quantities, procedures and operating arrangements of the services governed by this Code.

3. The following are inalienable infrastructural commons:

   a) the minimum care time necessary to guarantee levels 1 and 2 of the Basket;

   b) the essential care spaces of Circle 1 and their common equipment;

   c) the inventory of basic aids and non-clinical devices intended for community care;

   d) the essential knowledge of care and the related STVs and Safety Sheets;

   e) the aggregate balance of care time.

   The management of these goods shall not entail exclusive appropriation, rent or unjustified exclusion of the persons entitled.

4. Code 12 governs exclusively, within its own subject matter, the organisation of community care, the ascertainment of functional frailty, the Care Basket, care time and the internal function of care spaces. The following remain assigned:

   a) to Code 6, diagnosis, clinical services, clinical nutrition and other health acts;

b) to Code 9, the production and preparation of food and the technical and organisational management of collective canteens;

c) to Code C.1-Annex A, the manufacture and standardisation of aids;

d) to Code 10, the intended use and territorial siting of the spaces;

e) to Codes 1, 4 and 14, each for its own subject matter, the general provisions on the Basket, on guarantees and on safety.

5. This Code pursues the following objectives:

a) to guarantee the continuity of levels 1 and 2 of the Basket even in the presence of functional frailty;

b) to prevent the formation of rent on essential care services;

c) to ensure a fair, verifiable and sustainable distribution of care time;

d) to prevent isolation and unjustified recourse to forms of institutionalisation;

e) to promote, where appropriate and desired by the person, the recovery or maintenance of autonomy.

Art. 2 — Principles of community care

1. **Universality.** Every person has the right to the essential services of the Care Basket corresponding to levels 1 and 2, without being made subject to individual contribution to the CBO and in accordance with the guarantees provided for by Code 1.
2. **Autonomy of functional frailty.** Functional frailty constitutes a condition relevant for the purposes of community care, distinct from clinical diagnosis. Its ascertainment does not, in itself, require a health assessment. Clinical acts remain governed by Code 6.
3. **Proximity and continuity.** Essential care shall be organised, as far as possible, within Circle 1. Services that require competences or facilities not available locally shall be guaranteed through the federated network referred to in Article 6.
4. **Priority and scarcity.** The general priorities of the Basket are those established by Code 1, Article 32. In the event of insufficient care time, the reduction shall concern first the activities of a higher level, starting from level 5, in compliance with the hierarchy of Code 1 and without prejudice to the essential services of the lower levels. Within the applicable priorities, decisions shall take account of urgency, of the severity of the need and of the risk of interruption of care.
5. **Non-accumulation and non-monetisability.** Essential care time shall not be monetisable, nor shall it be individually accumulable to such an extent as to take availability away from the other persons entitled. Any technical limit on accumulation shall be defined by means of an STV, on the basis of the actual capacity of the service and with public criteria.
6. **Transparency and confidentiality.** The inventories of spaces and equipment, as well as the balances of care time, shall be published in aggregate and non-identifying form. The individual data necessary for the taking into care shall be processed exclusively for that purpose, in accordance with Code 7, Article 2.2.
7. **Reversibility and review.** Where the frailty is temporary, the individual care plan shall provide, if appropriate, for a pathway of support for autonomy and a time limit or condition for review. The review shall not result in the automatic interruption of essential services in the absence of a new assessment.
8. **Dignity, will and proportionality.** Services shall be organised with respect for the dignity, the preferences and the will of the person. Limitations of autonomy are permitted only in the cases and with the guarantees provided for by the applicable legal order and by the competent Codes.

Art. 3 — Care Council

1. Each Circle 1 shall establish a Care Council composed of three to five members, drawn by lot in

accordance with Article 12-bis of Code 3. Its composition shall ensure adequate competences, of at least level L2 or L3, in the field of community care, support for children from zero to three years of age, advanced old age, disability and the organisation of proximity services.

2. The members of the Council shall be drawn by lot in accordance with Article 12-bis of Code 3, shall remain in office for one year and shall not be drawn by lot again for the same Council before twelve months have elapsed from the end of their term of office. Replacement, the declaration of conflicts of interest and the minuting of decisions shall follow the procedures of Code 3.

3. The Care Council shall:

   a) propose and implement the Care Basket and coordinate the care spaces;

   b) draw up, update and apply the Safety Sheets relating to care activities, hygiene, early childhood and accompaniment;

   c) propose and update the Care STVs in accordance with the procedures of Code 2;

   d) plan the aggregate balance of care time and coordinate the shifts, with respect for individual freedom and for the competences of the participants;

   e) organise training at level L1 for basic care and at level L2 for the specialist activities provided for by Code 5;

   f) feed the Dependency Register governed by Code 15;

   g) coordinate the activities with the Health, Agroecological, Habitat, Production and Biodiversity Councils, each within its own competence.

4. The acts of the Care Council are divided into:

   a) **bound technical acts**, adopted in implementation of an STV or of a Safety Sheet in force;

   b) **discretionary proposals and decisions**, adopted in accordance with the consensus method and the procedures of Code 3, after publication for at least seventy-two hours, except in cases of reasoned urgency.

   Publication shall not include personal data or information that makes it possible to identify the persons taken into care.

5. The mandatory opinions of the competent Councils do not transfer ownership of the decision. The decision shall be taken jointly only where a Code expressly so provides.

6. In the absence of a Safety Sheet in force, the activity concerned shall not be started or continued in the ways that have not been assessed. The Care Council shall identify, where possible, a compliant substitute arrangement that ensures the continuity of levels 1 and 2. Every temporary derogation shall be reasoned, limited to what is strictly necessary and subject to verification.

Art. 4 — Care Basket and ascertainment of functional frailty

1. The Care Basket of levels 1 and 2 comprises, according to the need ascertained:

   a) the minimum care time;

   b) support during the meal, where the person is not autonomous in eating;

   c) assisted hygiene;

   d) essential accompaniment;

   e) protected rest;

   f) access to the space for children from zero to three years of age, where necessary.

2. The Care Basket is quantified principally in hours of assistance per person per week. The Care Basket STV shall define at least the services included, the minimum standards, the arrangements for daily distribution, the criteria of urgency, the response times and the review procedures. The standards shall not be reduced in such a way as to compromise levels 1 and 2 of the Basket.

3. The report of a need may be submitted by the person concerned or by anyone who detects a need for care. The report:

a) initiates the functional assessment;

b) allows, in the presence of an essential need, the provisional activation of the services necessary for a maximum period of seventy-two hours, pending the assessment;

c) shall not be used to initiate clinical treatments or to assign a diagnosis.

In the presence of an immediate risk to the life, health or physical safety of the person, the emergency procedures of the competent Codes shall be activated without delay.

4. The Care Council shall assess the functional condition within seventy-two hours of the report, on the basis of public criteria contained in the STV. The intervention of the Health Council is required only where health competences are necessary or where aspects of clinical risk emerge. The functional assessment does not constitute a diagnosis and shall not replace a health examination provided for by Code 6.

5. The individual care plan shall indicate at least:

a) the functional need detected;

b) the services and the frequency necessary;

c) the start date and the expected duration;

d) the reference person responsible for coordination;

e) the time limits and the criteria for review;

f) the arrangements for continuity or substitution in the event of unavailability of the service.

The person concerned shall take part in defining the plan with the appropriate means of communication and support. For a temporary frailty the review shall be carried out within ninety days, unless it needs to be brought forward.

6. The minimum list of the services of the Care Basket shall be approved by the Assembly on a proposal from the Care Council. The mandatory opinion of the Health Council is required, limited to the health aspects. The list, the standards and the procedures shall be published in the Care Basket STV.

7. The operational priorities shall apply in compliance with Code 1, Article 32, and in accordance with the following sequence, to be assessed in relation to urgency and individual need:

a) survival, essential hygiene and protected rest;

b) care of children from zero to three years of age and support for persons of advanced old age who are not self-sufficient;

c) convalescence and the period following childbirth;

d) support for the maintenance or recovery of autonomy;

e) further care services.

Art. 5 — Services, care time and coordination with the other Codes

1. Each Circle 1 shall ensure effective access to the following basic services, directly or through a shared network:

a) a kitchen supporting frail persons, functionally coordinated with the canteens of Code 9;

b) a common laundry;

c) a space for children from zero to three years of age;

d) a protected rest space;

e) proximity accompaniment.

The arrangements for provision shall be defined so as to guarantee accessibility, continuity and safety.

2. Circle 1 is responsible, collectively, for the availability of the total hours necessary to guarantee levels 1 and 2. Individual participation shall take place according to availability, competences and the rules of Code 3. Impositions incompatible with individual freedom, with the conditions of the person or with the competences required are not permitted. Individual unavailability shall not entail the loss of the frail

person’s right to essential services.

3. Care time shall be recorded in aggregate form in order to plan the services, verify the coverage of levels 1 and 2 and identify shortfalls. The recording shall not be used to profile the persons who receive or provide care outside the purposes permitted by Code 7.
4. Competences are allocated as follows:

   a) Code C.1-Annex A governs the manufacture, repair and standardisation of aids;

   b) Code 9 governs the production and preparation of food and the technical and organisational management of the canteens;

   c) Code 12 governs assistance to the frail person in access to and accompaniment at the meal, in assisted eating, in hygiene and in the daily use of aids.

   This Code does not assign to the Care Council the competence to produce aids or to organise the preparation of food.
5. For services of level 4 or 5, before new resources are requested, the possibility of reusing or recovering aids, clothing and spaces shall be verified, provided that the solution is safe, adequate to the need and does not reduce the quality of the essential services.
6. The right of elderly persons and persons who are not self-sufficient to remain in their own home is guaranteed, with community support in shifts included in the CBO, pursuant to Article 27 of the Constitution. Self-managed Common Houses are a choice of the person and shall not constitute a compulsory destination.

Art. 6 — Spaces, federated network and territory

1. Each Circle 1 shall activate and maintain the care spaces necessary, directly or through network agreements. The spaces shall be accessible, may be divided into functional areas and shall have up-to-date Safety Sheets for the activities actually carried out.
2. The intended use and the siting of the spaces are governed by Code 10. Code 12 governs the care function performed within them, without modifying the territorial designation or the constraints within the competence of Code 10.
3. The Confederation shall ensure at least one federated facility with advanced competences for each Basin composed of three or four Circles 1. Planning shall identify the size of the Basin, the services available and the arrangements for access. The facilities shall guarantee, according to need, competences in social paediatrics, support for complex disability and temporary emergency spaces.
4. Access to the federated facilities shall take place in accordance with criteria that are public, non-discriminatory and compatible with the urgency of the need. The inventory of resources shall be shared among the Circles concerned and updated in the manner established by the STV.
5. The measurement of services is limited to the data necessary for safety, the planning of care time and the verification of the Care Basket. Any individual data necessary for the taking into care shall remain confidential; public indicators shall be aggregate and non-profiling. Individual behavioural profiling not necessary for the purposes of this Code is prohibited, in conformity with Code 7, Article 2.2.

Art. 7 — Safety, training and right of interruption

1. No activity directed at frail persons may be started without a Safety Sheet that is in force and adequate to the activity. The Sheet shall be validated by the Care Council, after obtaining the mandatory opinion of the Health Council for the aspects relating to health risk, hygiene and assisted meals. Ownership of the validation remains with the Care Council, except in cases where a Code expressly provides for joint validation.
2. Care Safety Sheets shall be coordinated with the provisions of Code 1, Article 27, and of Code 14, each for its own competence. In the event of health or clinical risk the procedures of Code 6 shall apply.

3. The essential devices for community care shall be managed in the Health Storehouse governed by Code 6. They shall be available at no cost to the person concerned, subjected to maintenance and recorded in the relevant inventory. The unavailability of an essential device shall entail the suspension only of the activity that cannot be carried out safely; the Care Council shall activate, where technically possible, a compliant substitute function to ensure the continuity of levels 1 and 2.

4. The minimum training required is as follows:

   a) level L1 for operational access to the spaces for frail persons and for basic care activities;

   b) level L2 for activities involving children, persons of advanced old age who are not self-sufficient or persons with disabilities, in accordance with the classification of Code 5;

   c) level L3 for the design of care activities, the drafting of the STVs and the validation functions provided for by this Code.

5. Anyone who detects an immediate risk to a frail person shall interrupt the activity concerned, adopt the urgent protective measures permitted and inform the Care Council and the other competent Councils without delay. An interruption carried out in good faith in order to prevent harm shall not give rise to retaliation or detriment for the reporting person.

Art. 7-bis — Protection of minors and of temporarily incapable persons

1. In the event of violence, serious neglect or harm to the detriment of a minor or of a person who is unable to protect himself or herself, the Care Council shall activate without delay a protection pathway, pursuant to Article 26.3 of the Constitution. The pathway comprises making safe, listening and, if necessary, temporary reception with trusted persons or in a protected space.

2. The fact shall be reported to the Guarantee Circle for the procedures of Code 4. Reports in the Network shall be limited to the information necessary for protection. During the transition Article 37 of Code 4 shall apply.

3. The separation of a minor from his or her Circle 0 is permitted only as a temporary protection measure and shall be subject to validation by the Guarantee Tribunal within twenty-four hours, pursuant to Article 34 of Code 4.

4. Every person may designate in advance a representative for the event of incapacity, including by means of the living will provided for in Article 8-bis of Code 6.

5. In the absence of a designation and in the event of urgency, the Guarantee Circle shall appoint within twenty-four hours a temporary representative, chosen in the following order from among the persons of Circle 0 indicated by the person, his or her trusted person and a person drawn by lot with L2 competence in care, free of conflicts of interest. The office shall last no more than thirty days and shall be renewable by reasoned decision.

6. The representative shall decide in accordance with the expressed or presumable will of the person and, failing that, in accordance with his or her best interests. The office shall cease when the person recovers capacity.

Art. 8 — Care Dependency Register

1. The Care Council shall feed the Dependency Register governed by Code 15, indicating at least:

   a) **material dependencies**, relating to aids, devices or spaces that are not available in the Confederation, are available only beyond the critical time threshold defined in the STV or cannot be substituted locally;

   b) **competence dependencies**, relating to competences of level L2 or L3 not available in the Basin referred to in Article 6, paragraph 3;

   c) **external dependencies**, relating to care services external to the Confederation that are necessary to guarantee the continuity of levels 1 and 2.

2. For every critical dependency the Register shall indicate:

a) the missing resource or competence;

b) the service and the levels of the Basket concerned;

c) the person responsible for the plan to reduce the dependency;

d) the temporary measures of continuity and risk mitigation;

e) the final time limit laid down for overcoming the dependency or for a new reasoned assessment.

3. The exit plan shall provide for a definite final time limit. Where overcoming the dependency is not technically achievable within that time limit, the Care Council shall propose a reasoned revision of the plan and of the continuity measures; the revision shall not be limited to extending the dependency without indicating interventions and responsibilities.

4. Every year a federated audit of the autonomy of care shall be carried out by independent L3 members, drawn by lot in accordance with Article 36 of Code 4. The audit shall also assess the actual coverage of levels 1 and 2 and respect for confidentiality.

Art. 9 — Prohibitions, guarantees and remedies

1. It is prohibited to:

a) derive rent from the Care Basket of levels 1 and 2, from the essential spaces, from the minimum care time or from the essential devices;

b) make access to the life-saving STVs and to the Safety Sheets subject to payment or to an exclusive licence;

c) reserve exclusively or accumulate care time, spaces or aids beyond the technical limits, where this reduces availability for other persons entitled;

d) deny or suspend an essential service of the Care Basket as a sanctioning measure;

e) transfer to external parties the control of the essential care spaces;

f) use data on functional frailty for profiling or for purposes other than the taking into care and the technical purposes expressly permitted by Code 7;

g) order the forced institutionalisation of a person, except for a temporary placement strictly necessary in the face of an immediate risk, adopted with the guarantees of Code 4. Any health treatments remain governed by Code 6 and by the related guarantees.

2. Violations of paragraph 1 constitute substantial violations. The ascertainment, the precautionary measures, the suspension or deactivation of the function and the remedies shall be adopted in accordance with Code 4. The Care Council shall document the facts and activate the procedures provided for, without taking the place of the bodies competent for the ascertainment or for the application of the remedies.

3. The suspension or deactivation of a non-compliant function shall not interrupt the services of levels 1 and 2 where it is possible to guarantee them through a technically compliant substitute function. In the event of immediate risk, the urgent measures provided for by the competent Codes shall apply, with simultaneous activation of continuity of care as far as possible.

4. The person concerned or anyone who demonstrates a qualified interest may appeal to the Guarantee in accordance with Code 4, within fourteen days of the communication of the decision or of actual knowledge of the violation. The appeal may concern procedural defects, manifest technical unreasonableness, discrimination or disproportion. The appeal shall not automatically suspend the urgent measures necessary to prevent harm, unless the competent body decides otherwise.

Art. 10 — Implementation, coordination and precedence

1. This Code is coordinated, for the respective subject matters, with:

a) Code 1, Articles 27 and 32, for the Safety Sheets and the order of priority of the Basket;

b) Code 2, for the STVs;

c) Code 3, for resolutions and the consensus method;

d) Code 4, for the Guarantee and the remedies;

e) Code 5, for the levels of competence and training;

f) Code 6, for health, clinical services and the Health Storehouse;

g) Code 7, Article 2.2, for the rules on the network and on data;

h) Code 8, for any water resources necessary for the care spaces;

i) Code C.1-Annex A, for aids;

j) Code 9, for food production and the canteens;

k) Code 10, for the territorial designation of the spaces;

l) Codes 13, 14 and 15, for their respective subject matters, including biodiversity, safety and the Dependency Register.

2. Within six months of entry into force, each Circle 1 shall:

a) inventory, in a manner respectful of confidentiality, the functional needs relevant to the continuity of levels 1 and 2;

b) inventory the care spaces, the equipment and the time available;

c) activate the minimum Care Basket by means of a provisional STV and minimum Safety Sheets.

3. Within twelve months of entry into force, each Circle 1 shall:

a) establish the Care Council;

b) guarantee access to the minimum care spaces;

c) activate the federated inventory of aids;

d) adopt the definitive STVs for levels 1 and 2;

e) identify its own node of the federated Care Network.

In the presence of documented overlaps of the transitional burdens relating to Codes 9, 10 and 12, the Assembly may grant a reasoned extension, for a period not exceeding six months. The extension shall indicate the activities still to be completed, the persons responsible and the intermediate deadlines and shall not suspend the guarantee of levels 1 and 2.

4. Existing functions found to be in violation of Article 9 shall be subject without delay to verification in accordance with Code 4. Within ninety days of entry into force there shall be initiated, for each function concerned, a documented compliance pathway or, if compliance is technically impossible, the deactivation procedure provided for by Code 4. The plan shall identify the substitute function and a definite final time limit. The deactivation of a function that guarantees levels 1 and 2 shall take place only after the activation of the substitute, except for the urgent measures necessary to deal with an immediate risk, adopted with the guarantees of Code 4.

5. In the event of regulatory overlap, each Code prevails only as regards the subject matter assigned to it:

a) Code 12 for the organisation of community care, the ascertainment of functional frailty, the Care Basket and the internal function of care spaces;

b) Code 6 for diagnosis, clinical services, clinical nutrition and health risks;

c) Code 9 for the production and preparation of food and the technical and organisational management of the canteens;

d) Code 10 for the intended use and territorial siting of the spaces;

e) Code C.1-Annex A for the manufacture and standardisation of aids;

f) Code 14 for the safety aspects within its competence.

Precedence is limited to the specific subject and does not entail the absorption of the competences assigned to the other Codes. In the event of a conflict that cannot be resolved by coordinated interpretation, the competent bodies shall define a joint solution in accordance with the procedures of the legal order, ensuring in the meantime the protection measures necessary and the continuity of levels 1 and 2.